

Amendment No. 1 to HB2572

Terry
Signature of Sponsor

AMEND Senate Bill No. 2279*

House Bill No. 2572

by deleting all language after the enacting clause and substituting:

SECTION 1. Tennessee Code Annotated, Section 68-1-128(c)(1), is amended by deleting the subdivision and substituting:

(1) In addition to identifying prescribers under subsections (a) and (b), beginning July 1, 2017, and annually thereafter, the department shall identify high-risk prescribers based on clinical outcomes, including patient overdoses. The department shall determine the criteria for identifying a provider as a high-risk prescriber and whether a provider is a high-risk prescriber. The department shall make the criteria for identifying a provider as a high-risk prescriber publicly available on the department's website. A provider identified as a high-risk prescriber under this subdivision (c)(1) is subject to selected chart review and investigation by the department.

SECTION 2. Tennessee Code Annotated, Section 68-1-128(c)(3)(D), is amended by deleting "four-week" and substituting "thirty-day".

SECTION 3. Tennessee Code Annotated, Section 68-1-128(c), is amended by adding the following as a new subdivision:

(7) The department shall expunge a prescriber's identification as a high-risk prescriber under subdivision (c)(1) and notify the prescriber in writing of the expungement upon receiving proof that the prescriber has completed the course requirements of subdivision (c)(3)(A) and either:

(A) None of the patient overdoses that formed the basis for the prescriber's identification as a high-risk prescriber involved a substance prescribed by the prescriber; or

(B) The department does not identify the prescriber as a high-risk prescriber for the next two (2) consecutive years.

SECTION 4. Tennessee Code Annotated, Section 63-1-306, is amended by inserting the following as a new subsection (c), and redesignating the existing subsection (c) and remaining subsections accordingly:

(c)

(1) A pain management specialist may provide coverage for and serve as medical director in place of a medical director who is temporarily unable to fulfill the medical director's duties because of illness, vacation, or other unavailability. The coverage may be provided on a temporary, short-term basis and does not require the in-person presence of the pain management specialist providing the coverage if that pain management specialist is immediately available by telephone.

(2) Temporary service as a medical director under subdivision (c)(1) does not count toward the limit of four (4) pain management clinics at which a pain management specialist may serve as medical director under § 63-1-309(c).

(3) The department shall not require a medical director to notify the department of the identity of another pain management specialist serving as medical director under subdivision (c)(1) during the medical director's absence.

SECTION 5. Tennessee Code Annotated, Section 63-1-309(c), is amended by deleting "weekly" and substituting "quarterly".

SECTION 6. Tennessee Code Annotated, Section 63-1-316, is amended by adding the following as a new subsection:

(k) The commissioner or the commissioner's designee may issue an advisory private letter ruling to an affected licensee that requests a ruling on a matter within this part. A private letter ruling issued under this subsection (k) affects only the licensee that requested the ruling and has no precedential value for another inquiry or a future contested case. A dispute regarding a private letter ruling may, at the election of the commissioner or the commissioner's designee, be resolved under § 4-5-223.

SECTION 7. Tennessee Code Annotated, Section 63-1-316(c), is amended by adding the following to the end of the subsection:

The department shall make all inspection criteria required for compliance by pain management clinics available to the public on the department's website.

SECTION 8. This act takes effect upon becoming a law, the public welfare requiring it.